ADR Reporting & Documentation

How to complete a structured ADR reporting form from a case scenario, with a worked example

PHARMACOVIGILANCE INTERNSHIP | TASK 2

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Goal of this task: turn a case scenario into a complete, accurate and consistent ADR report. This guide explains what each part of the form asks for, gives a step-by-step workflow, and shows a **fully completed sample form** (fictional case) you can use as a model.

1. ADR Reporting: The Basics

- **Why report:** spontaneous (voluntary) reporting is the backbone of post-marketing drug safety. It picks up rare, delayed and serious reactions that clinical trials miss.
- **Who reports:** doctors, pharmacists, nurses and patients; companies (marketing authorisation holders, MAHs) must also forward the reports they receive to the regulator.
- **What to report:** any *suspected* ADR, especially serious ones, reactions not listed in the label, reactions to new drugs, and reactions in children, pregnancy or the elderly. **Suspicion is enough**; you do not need proof that the drug caused it.
- **Report one case at a time:** each patient's report is an Individual Case Safety Report (ICSR).

> **Minimum criteria for a valid report (hook: 2 I's, S, E)**
>
> **I**dentifiable patient, **I**dentifiable reporter, **S**uspect drug, **E**vent. If any one is missing, the report is incomplete and follow-up must be requested.

Timelines. Serious cases are reported faster than non-serious ones. As a general rule, a company sends serious, unexpected reports to the regulator within 15 calendar days of first receiving the minimum information. Deadlines differ by region and case type, so always follow local rules and your SOP.

2. Structure of a Standard ADR Reporting Form

Common forms (such as CIOMS I, FDA MedWatch and national suspected-ADR forms) share the same core blocks. The labels and order vary, but the content is the same.

Section	What it captures
A Patient information	Initials or code, age or date of birth, sex, weight.
B Suspected adverse reaction	Description, onset and end dates, seriousness, treatment given, outcome, test results.
C Suspect medication(s)	Drug name, batch, manufacturer, dose, route, frequency, indication, dates, action taken, dechallenge and rechallenge.
D Concomitant drugs and history	All other medicines (including OTC and herbal), relevant diseases, allergies, habits.
E Reporter details	Name, profession, contact details, date of report.
F Assessment	Causality, expectedness and seriousness. Often completed by the safety reviewer.

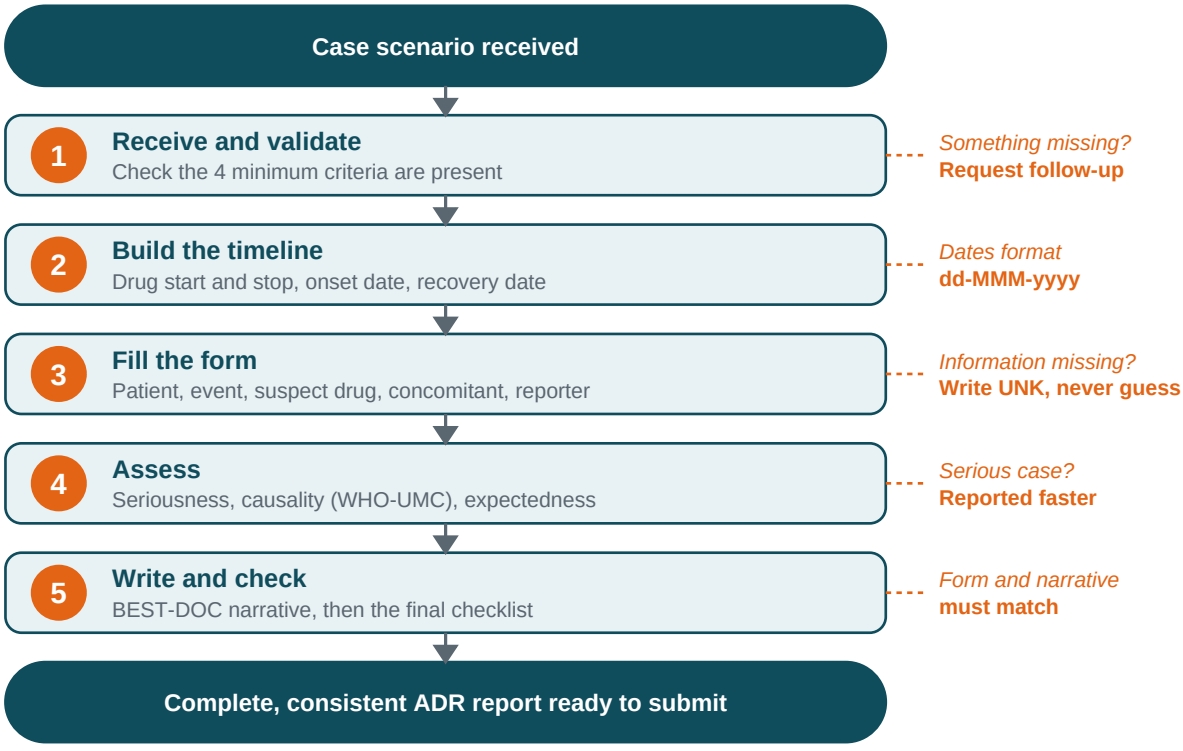
3. Field-by-Field Guide

Field	How to complete it	Watch out for
Patient identifier	Initials or a code. Never the full name.	Do not add address or phone number.
Age, sex, weight	Age at onset (or date of birth); weight in kg.	If unknown, write UNK. Do not guess.

Field	How to complete it	Watch out for
Reaction description	Reporter's own words plus the diagnosis, with signs, onset, course and outcome.	Vague labels such as 'allergic' without describing the signs.
Dates	Use dd-MMM-yyyy (for example 03-Sep-2026) for start, stop, onset and recovery.	Onset before the drug start date means you must recheck.
Seriousness	Tick the criterion: death, life-threatening, hospitalisation, disability, congenital anomaly, other medically important.	Severity is not seriousness.
Outcome	Recovered, recovering, not recovered, recovered with sequelae, fatal, or unknown.	Must match the narrative.
Suspect drug	Generic name (and brand), batch, manufacturer, dose, frequency, route, indication, start and stop.	Mixing up dose per intake with dose per day.
Action taken	Drug withdrawn, dose reduced, dose unchanged, or not applicable.	Link it to the dechallenge result.
Dechallenge / rechallenge	Positive, negative, or not done / not applicable.	Note if other treatment was also given, as it can confuse the result.
Concomitant drugs	Every other medicine with dose and dates, including OTC, herbal and vaccines.	Leaving them out is a common gap.
Medical history	Relevant diseases, allergies, pregnancy, alcohol, smoking, earlier reactions.	Only what is relevant.
Reporter	Name, profession, contact, date of report.	Needed for follow-up questions.

4. Workflow: From Case Scenario to Completed Form

Follow these steps in order for any case scenario you are given.



Writing the narrative: BEST-DOC

Letter	Include
B Background	Age, sex, weight, relevant history, concomitant drugs.
E Event	What happened, when it started, signs and test results, in time order.
S Suspect drug	Name, dose, route, indication, start date (and stop date).
T Treatment	What was done for the reaction (drugs, procedures, hospital care).
D Dechallenge / rechallenge	What happened when the drug was stopped, and whether it was restarted.
O Outcome	Recovered, recovering, not recovered, sequelae or fatal, with the date.
C Causality	Reviewer's assessment (WHO-UMC and/or Naranjo) with a one-line reason.

5. Good Documentation Principles

Safety reports are legal and scientific records. Follow **ALCOA**:

Principle	Meaning in an ADR report
A Attributable	It is clear who recorded the data and who reported it.
L Legible	Clear, readable text; standard dates; no unexplained abbreviations.
C Contemporaneous	Recorded promptly, with the date the report was received.
O Original	Matches the source document (or a certified copy).
A Accurate	Correct, complete and consistent between the form and the narrative.

Handling missing information: write **UNK** (unknown), **NA** (not applicable) or **ND** (not done). Never leave a required field blank and never invent a value.

6. Completed Sample Form (Fictional Case)

Case scenario

A 62-year-old woman (68 kg) with knee osteoarthritis and hypertension has taken telmisartan 40 mg once daily for three years. On 03-Sep-2026 her orthopaedic doctor started oral diclofenac sodium 50 mg twice daily for knee pain. On 10-Sep-2026 she developed epigastric pain and black tarry stools, and on 12-Sep-2026 she felt dizzy and was admitted to hospital. Haemoglobin was 8.2 g/dL (12.5 g/dL on 01-Aug-2026). Endoscopy showed a bleeding gastric ulcer; the H. pylori test was negative. She had no previous ulcer, no alcohol use and no other NSAID or anticoagulant use. Diclofenac was stopped on 12-Sep-2026. She received IV pantoprazole and 2 units of packed red cells, the bleeding settled, and she was discharged on 17-Sep-2026.

SECTION A: PATIENT INFORMATION	
Patient initials / ID	M.S. / PT-001
Age / Sex / Weight	62 years / Female / 68 kg

SECTION B: SUSPECTED ADVERSE REACTION	
Reaction (reported)	Epigastric pain and black tarry stools, followed by dizziness.
Diagnosis / suggested term	Bleeding gastric ulcer (suggested MedDRA term: Gastric ulcer haemorrhage).
Onset date	10-Sep-2026 (7 days after starting the suspect drug)
Date of recovery / outcome	Recovered; discharged 17-Sep-2026
Seriousness	**Serious**: hospitalisation. Severity: severe.
Treatment for reaction	Drug stopped; IV pantoprazole; 2 units packed red cells.
Relevant tests	Hb 8.2 g/dL on admission (baseline 12.5 g/dL on 01-Aug-2026). Endoscopy: bleeding gastric ulcer. H. pylori: negative.

SECTION C: SUSPECT MEDICATION	
Drug (generic / brand)	Diclofenac sodium / Brand: UNK
Batch no. / Manufacturer	UNK / UNK
Dose, frequency, route	50 mg twice daily (100 mg/day), oral
Indication	Knee osteoarthritis pain
Therapy dates	Start 03-Sep-2026; Stop 12-Sep-2026 (9 days)
Action taken	Drug withdrawn
Dechallenge	Positive (bleeding settled after stopping; ulcer treatment was also given)
Rechallenge	Not done

SECTION D: CONCOMITANT DRUGS AND HISTORY	
Concomitant drug	Telmisartan 40 mg once daily, oral, for hypertension; ongoing for about 3 years.
Relevant history	Knee osteoarthritis; hypertension. No previous peptic ulcer or GI bleed. No alcohol. No other NSAID or anticoagulant.

SECTION E: REPORTER DETAILS	
Reporter	Dr. A. Sharma (example), Consultant Gastroenterologist
Contact / Date of report	As per site record / 21-Sep-2026

SECTION F: ASSESSMENT	
WHO-UMC causality	**Probable / Likely** (diclofenac)
Naranjo score	Q1 +1, Q2 +2, Q3 +1, Q4 0, Q5 +2, Q6 to Q9 0, Q10 +1 = **7 (Probable)**
Type / Expectedness	Type A (dose-related, predictable). Listed in the product label (expected).
Reason	Plausible timing, known NSAID effect, no other cause found, improved after withdrawal, confirmed on endoscopy.

Sample narrative (BEST-DOC)

A 62-year-old female patient (68 kg) with knee osteoarthritis and hypertension, on telmisartan 40 mg daily for about three years, was started on oral diclofenac sodium 50 mg twice daily on 03-Sep-2026 for knee pain. She had no history of peptic ulcer, alcohol use, or other NSAID or anticoagulant use. On 10-Sep-2026, seven days after starting diclofenac, she developed epigastric pain and black tarry stools, followed by dizziness on 12-Sep-2026. She was admitted to hospital with a haemoglobin of 8.2 g/dL (12.5 g/dL on 01-Aug-2026). Endoscopy showed a bleeding gastric ulcer; H. pylori testing was negative. Diclofenac was stopped on 12-Sep-2026. She received IV pantoprazole and 2 units of packed red cells, the bleeding settled, and she was discharged on 17-Sep-2026, recovered. No rechallenge was done. The event is serious (hospitalisation). Causality is Probable/Likely (WHO-UMC) and Naranjo score is 7.

7. Final Checklist Before Submitting

Check	Done
All 4 minimum criteria present (patient, reporter, suspect drug, event)	[ ]
Patient identified by initials or code only	[ ]
Dates in one format (dd-MMM-yyyy); onset is after the drug start date	[ ]
Suspect and concomitant drugs separated; dose, route and dates complete	[ ]
Seriousness criterion ticked; severity not confused with seriousness	[ ]
Action taken, dechallenge and rechallenge filled in	[ ]
Outcome stated and matches the narrative	[ ]
Missing items written as UNK / NA / ND, nothing guessed	[ ]
Narrative is in time order and matches every field on the form	[ ]
Causality category and short reason recorded	[ ]

The sample case is fictional and for training only. For a real report, use the form and SOP provided by your organisation and follow current regulatory requirements (for example ICH E2B, CIOMS, and local rules such as PvPI in India or MedWatch in the US).